Society of Regional Anesthesia (ASRA) Pain Medicine Consensus practice guidelines on interventions for lumbar facet joint pain from a multispecialty, international working group make the following recommendations:

- Medial branch blocks should be the prognostic screening test of choice before lumbar facet RFA.
- Repeat RFA procedures for recurrence of pain are recommended in patients who experienced a good outcome from the first RFA procedure, typically defined as at least 50% relief of pain at 3 months.
- Given the drop-off in success rates reported in some studies and the mean duration of benefit, the guidelines recommend repeating the procedure no more than two times per year.

National Institute for Health and Care Excellence (NICE) NICE guidelines (2016; updated 2020) on the management of LBP and sciatica make the following recommendations:

- Consider referral for assessment for radiofrequency denervation for people with CLBP when:
  - Non-surgical treatment has not worked for them; and
  - The main source of pain is thought to come from structures supplied by the medial branch nerve; and
  - They have moderate or severe levels of localized back pain (rated as 5 or more on a visual analog scale, or equivalent) at the time of referral.
- Only perform radiofrequency denervation in people with CLBP after a positive response to a diagnostic medial branch block.
- Do not offer imaging for people with LBP with specific facet joint pain as a prerequisite for radiofrequency denervation.

North American Spine Society (NASS) NASS clinical guidelines provide evidence-based recommendations which are endorsed by the American Academy of Physical Medicine and Rehabilitation (AAPM&R) and American Association of Neurological Surgeons and Congress of Neurological Surgeons (AANS/CNS), for the diagnosis and treatment of adults with LBP. The guidelines make the following recommendations regarding RFA:

- Thermal RFA is suggested as a treatment for patients with LBP from the zygapophyseal joints. The outcomes of this procedure become more reliable when more stringent diagnostic criteria are used. The relief from these ablations is durable for at least six months following the procedure. Grade of recommendation: B - fair evidence (Level II or III studies with consistent findings) for or against recommending intervention.
- Cooled RFA of the sacral lateral branch nerves and dorsal ramus of L5 may be considered in patients with sacroiliac joint pain diagnosed with dual diagnostic blocks. Grade of recommendation: C - poor quality evidence (Level IV or V studies) for or against recommending intervention.
- There is insufficient evidence to make a recommendation for or against the use of cryodenervation for the treatment of zygapophyseal joint pain. Grade of recommendation: I - insufficient or conflicting evidence not allowing a recommendation for or against intervention.

International Society for the Advancement of Spine Surgery (ISASS)